National Cancer Control Programme (NCCP) 00900 · clinician draft

Nivolumab + ipilimumab · first-line colorectal

Colorectal treatment setting First-line unresectable or metastatic MSI-H/dMMR colorectal cancer. Other nivolumab/ipilimumab regimens have different doses, routes and timing.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycles 1–4: nivolumab + ipilimumab every 21 days

Active medicines: Nivolumab + Ipilimumab

Emetogenic risk: Low combination; nivolumab minimal, ipilimumab low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before a combination treatment day, if prescribed under the low-risk antiemetic plan NCIS V8 low-risk reference, distinct from infusion premedication. Review QT risk, electrolytes and interactions.
Metoclopramide	10 mg orally up to three times daily as prescribed	As-needed rescue on subsequent days Confirm contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Routine infusion premedication is not usually required. Antiemetics above are a reference requiring individual/local prescription; do not add prophylactic corticosteroids automatically.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.
- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Confirm nivolumab first, separate infusion sets/filters and source-specific observation. The weight-based 00551 regimen and flat-dose 00900 regimen are not interchangeable.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Cycle 5 onwards: prescribed nivolumab monotherapy schedule

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or infusion premedication; prescribe symptom relief individually.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.
- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Cycles 1–4: nivolumab 240 mg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg intravenous (IV) every two weeks or 480 mg IV every four weeks.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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